



## DISCHARGE SUMMARY

Department of General Medicine

|                   |                       |                   |                         |
|-------------------|-----------------------|-------------------|-------------------------|
| Patient Name      | Meera Iyer            | UHID              | SMH-208841              |
| Age / Sex         | 61 Years / Female     | IP Number         | IP-26-04417             |
| Date of Admission | 22/09/2026 10:40      | Date of Discharge | 27/09/2026              |
| Consultant        | Dr. Anil Kulkarni, MD | Ward / Bed        | Medical Ward 3 / Bed 12 |

### DIAGNOSES

| Diagnosis                                               | ICD-10 | Status                  |
|---------------------------------------------------------|--------|-------------------------|
| Diagnosis: Type 2 diabetes mellitus with hyperglycaemia | E11.65 | Principal, active       |
| Diagnosis: Chronic kidney disease stage 3a              | N18.31 | Secondary, chronic      |
| Diagnosis: Essential hypertension                       | I10    | Secondary, chronic      |
| Diagnosis: Hypothyroidism                               | E03.9  | Secondary, on treatment |
| Diagnosis: Dyslipidaemia                                | E78.5  | Secondary, on treatment |

### PRESENTING COMPLAINTS

61 year old woman, known diabetic for 12 years and hypertensive for 9 years, presented with 3 days of increased thirst, frequent urination, fatigue and giddiness. No chest pain, fever or vomiting. Home fasting sugars had been 220 to 280 mg/dL over the past week.

### PAST HISTORY

Type 2 diabetes since 2014, hypertension since 2017 and hypothyroidism since 2019. Chronic kidney disease was noted on routine labs in 2025. No known drug allergies. Non-smoker, no alcohol.

### EXAMINATION ON ADMISSION

|                               |                   |                   |                       |                 |
|-------------------------------|-------------------|-------------------|-----------------------|-----------------|
| BLOOD PRESSURE<br>148/92 mmHg | PULSE<br>84 / min | SpO2<br>98 % (RA) | TEMPERATURE<br>98.4 F | WEIGHT<br>68 kg |
|-------------------------------|-------------------|-------------------|-----------------------|-----------------|

Mild dehydration. Chest clear, heart sounds normal, abdomen soft. No pedal oedema.

### ALLERGIES AND ALERTS

No known drug allergies. Renal impairment: avoid NSAIDs and review doses of renally cleared medicines. Glimepiride carries a risk of low sugars: counselled on symptoms and on carrying glucose.



HOSPITAL COURSE AND INVESTIGATIONS

Meera Iyer | UHID SMH-208841

HOSPITAL COURSE

Admitted with uncontrolled hyperglycaemia. Intravenous fluids and a basal-bolus insulin schedule were started and blood sugars settled to 140 to 180 mg/dL by day 3. Renal function was monitored daily; creatinine improved with hydration and was stable at discharge. Potassium was mildly raised on admission and was managed with diet. Urine routine showed no infection. Insulin was stopped on day 4 and oral agents were resumed.

LABORATORY INVESTIGATIONS

Sample collected: 22/09/2026 11:30 (on admission)

| Test            | Result | Flag | Unit           | Reference |
|-----------------|--------|------|----------------|-----------|
| HbA1c           | 8.4    | H    | %              | < 7.0     |
| Fasting glucose | 164    | H    | mg/dL          | 70 - 100  |
| Creatinine      | 1.5    | H    | mg/dL          | 0.6 - 1.3 |
| eGFR            | 47     | L    | mL/min/1.73 m2 | > 60      |
| Potassium       | 5.2    | H    | mmol/L         | 3.5 - 5.0 |
| Sodium          | 136    |      | mmol/L         | 135 - 145 |

Repeat sample collected: 27/09/2026 07:50 (before discharge)

| Test       | Result | Flag | Unit   | Reference |
|------------|--------|------|--------|-----------|
| Creatinine | 1.4    | H    | mg/dL  | 0.6 - 1.3 |
| Potassium  | 4.9    |      | mmol/L | 3.5 - 5.0 |

OTHER INVESTIGATIONS

- ECG: sinus rhythm at 82/min, no acute ST-T changes.
- Chest X-ray: normal cardiac size, clear lung fields.
- Ultrasound abdomen: both kidneys normal in size with mildly increased echogenicity.
- Urine routine: glucose 3+, no protein, no pus cells.



DISCHARGE MEDICATION AND FOLLOW UP

Meera Iyer | UHID SMH-208841

MEDICINES ON DISCHARGE

| # | Medicine          | Strength | Dose and frequency | Remarks          |
|---|-------------------|----------|--------------------|------------------|
| 1 | Tab Metformin     | 500 mg   | Twice daily        | After meals      |
| 2 | Tab Glimepiride   | 1 mg     | Once daily         | Before breakfast |
| 3 | Tab Telmisartan   | 40 mg    | Once daily         | Morning          |
| 4 | Tab Amlodipine    | 5 mg     | Once daily         | Morning          |
| 5 | Tab Atorvastatin  | 20 mg    | Once daily         | At night         |
| 6 | Tab Levothyroxine | 75 mcg   | Once daily         | Empty stomach    |
| 7 | Tab Pantoprazole  | 40 mg    | Once daily         | Before breakfast |

ADVICE ON DISCHARGE

- Diabetic diet with low salt and low potassium foods. Walk 30 minutes daily. Drink 2 litres of fluid a day.
- Check fasting and post-meal sugars at home and bring the record to review.
- Avoid painkillers of the NSAID group such as diclofenac and ibuprofen because of the kidney disease.
- Return at once for vomiting, breathlessness, very low sugars, reduced urine or swelling of the feet.

FOLLOW UP

Review in Medicine OPD in 1 week with HbA1c, creatinine, potassium and TSH reports. Nephrology opinion advised if creatinine rises above 1.8 mg/dL.

A. Kulkarni

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